INDITECH HEALTH

Solution proposal

Bro-Zedex

A comprehensive clinic-embedded program to transform pediatric cough management

1) Program Overview

Program name

Cough Assistant Program (Clinic Physical + Digital Ecosystem)

A clinic-embedded cough triage + education + follow-up system, supported by an academy (SAPA), designed to help pediatricians manage cough safely and consistently, while enabling the brand to appear **only on doctor-facing touchpoints** at the correct clinical decision moment.

Why this program will work

Bro-Zedex LS is in a crowded wet cough market. Broad cough education helps everyone (especially the category leader). So instead of "more education meetings," we build something that **lives inside clinic workflow** and gets used for every cough patient: a clinic-branded **Cough Assistant** screening + red-flag triage + patient education service, and a doctor-side report that carries a simple algorithm and (where clinically appropriate) the **Bro-Zedex LS** brand cue.

Duration & scale

- **Duration: April–December 2026** (field program)

> Note: April–Dec is **9 calendar months**. To deliver **10 case-study publications**, we will do **two publications in the first month** (suggested: April kickoff month) OR treat **March 2026 as "Month 0: onboarding"** and still keep active roll-out April–Dec.

- **Target HCPs: ~2,000 pediatricians**
- **Field mapping: ~400 ASMs, ~5 HCPs/ASM**

What the pediatrician gets (value-first)

1. **A clinic-branded cough screening + red-flag triage link/QR** parents can use anytime (no app download needed).
2. **Local-language parent education videos** matched to the case-study theme.
3. **Doctor-side cough report for children showing red flags by email** with:
   - child's screening summary
   - red flags
   - "at-a-glance" cough algorithm for quick decision-making
4. **A doctor certification course**: "Chesty Cough and Wheeze in Children: Red-Flag Triage and Rational Levosalbutamol Use in Everyday Practice"
5. **Monthly case-study publications** (10) for practical learning and clinic reinforcement.

2) Program Components

A. Physical clinic layer (what the ASM helps install/activate)

1. External clinic display board

Copy provided in Section 5 below.

2. Waiting-room poster (inside clinic)

- Headline: "No more anxiety when your child has cough. Scan this QR code and use the link whenever you child develops cough for a quick check & guidance."
- QR → opens the clinic's Cough Assistant link (

- multilingual).

3. Monthly PDF share (Case Study Publication)

- Given by ASM in every monthly call.
- Each insert includes **QR/link to the matching parent education video** (doctor or clinic staff can share).
- Last page of the PDF is an advertisement section reserved for brand communication.

Digital parent layer

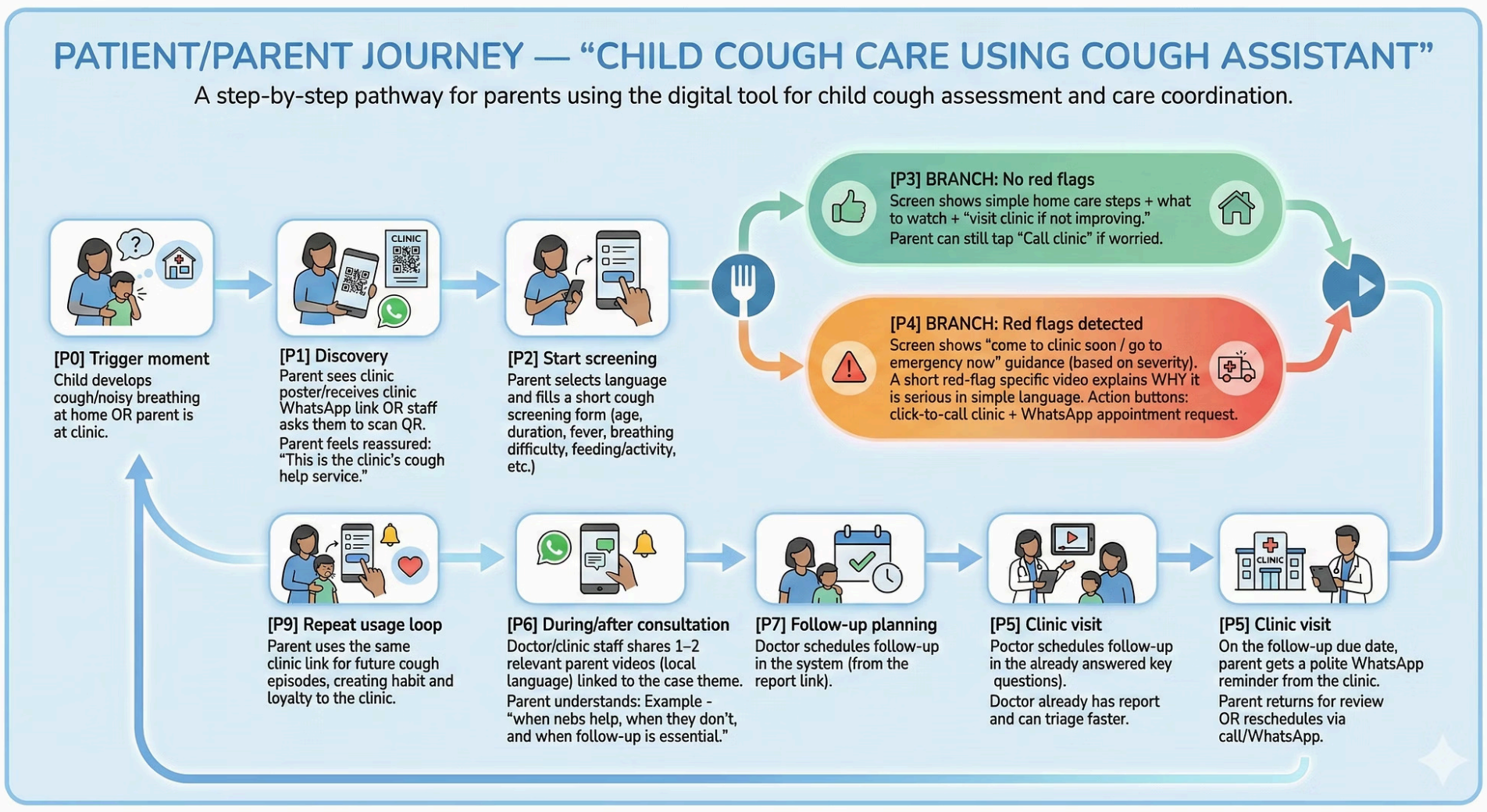
B. Digital parent layer (what parents use)

The parent-facing experience is:

- **Clinic-branded and comforting**
- **Multilingual**
- **No brand name / no medicine advice** Everything is positioned as a clinic service.

Parent flow inside Cough Assistant (core screens):

- Start → short cough screening form (age, duration, fever, breathing difficulty, noisy breathing, fast breathing, feeding/activity, etc.)
- Output screen:
  - **No red flags:** home-care guidance + when to visit clinic
  - **Red flags:** Visit clinic/urgent visit guidance + red-flag specific video + action buttons
- Action buttons:
  - click-to-call clinic
  - WhatsApp "appointment request" message button



Doctor-facing tools

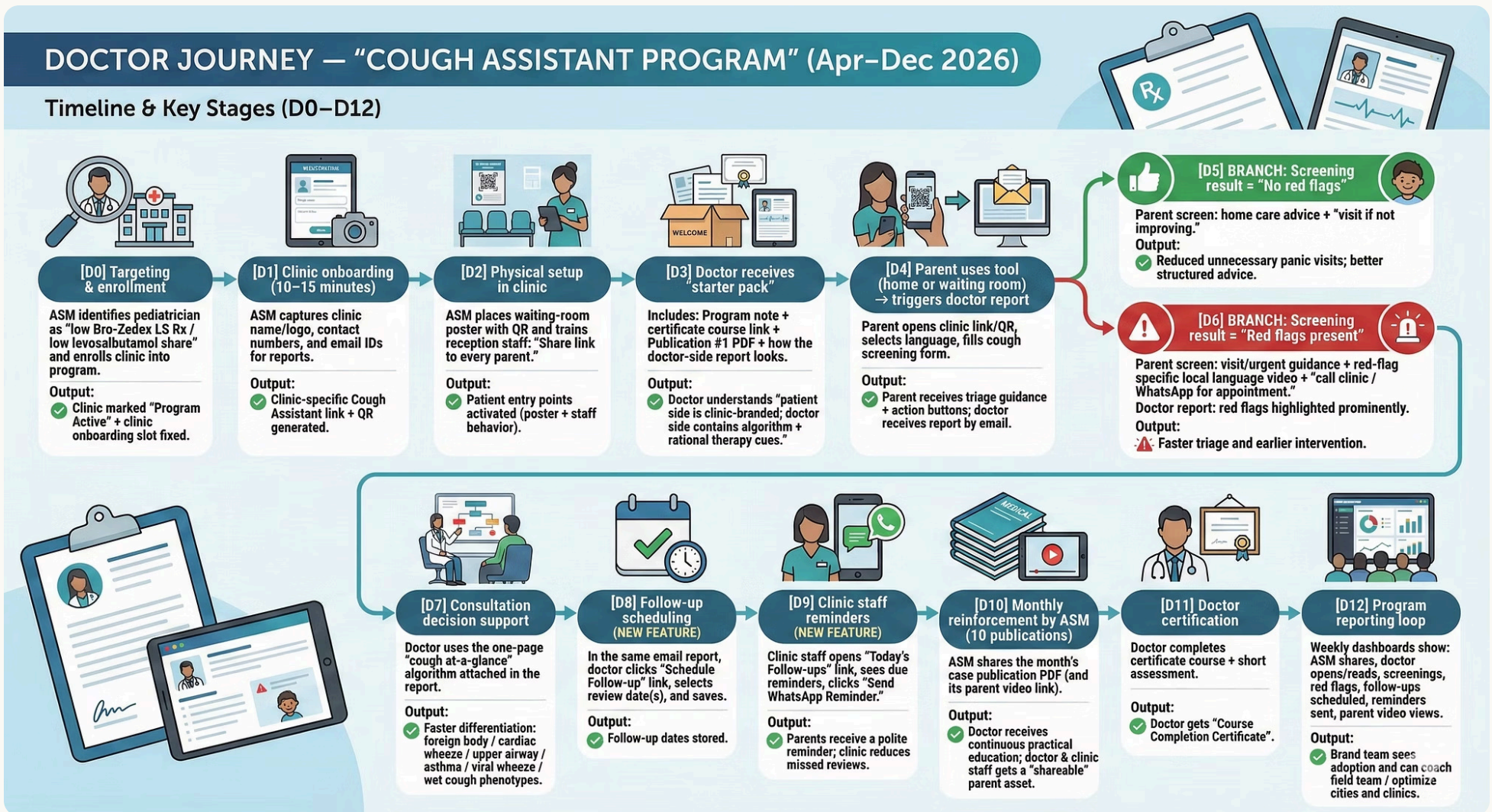
C. Doctor layer (what the pediatrician receives)

Every time a parent submits the form and red flags are found:

- The clinic receives a **Cough Assistant report** by **email**.
- The report includes:
  - child details and answers
  - red flags highlighted with at-a-glance guidance
  - **one-page cough algorithm** for rapid triage/management

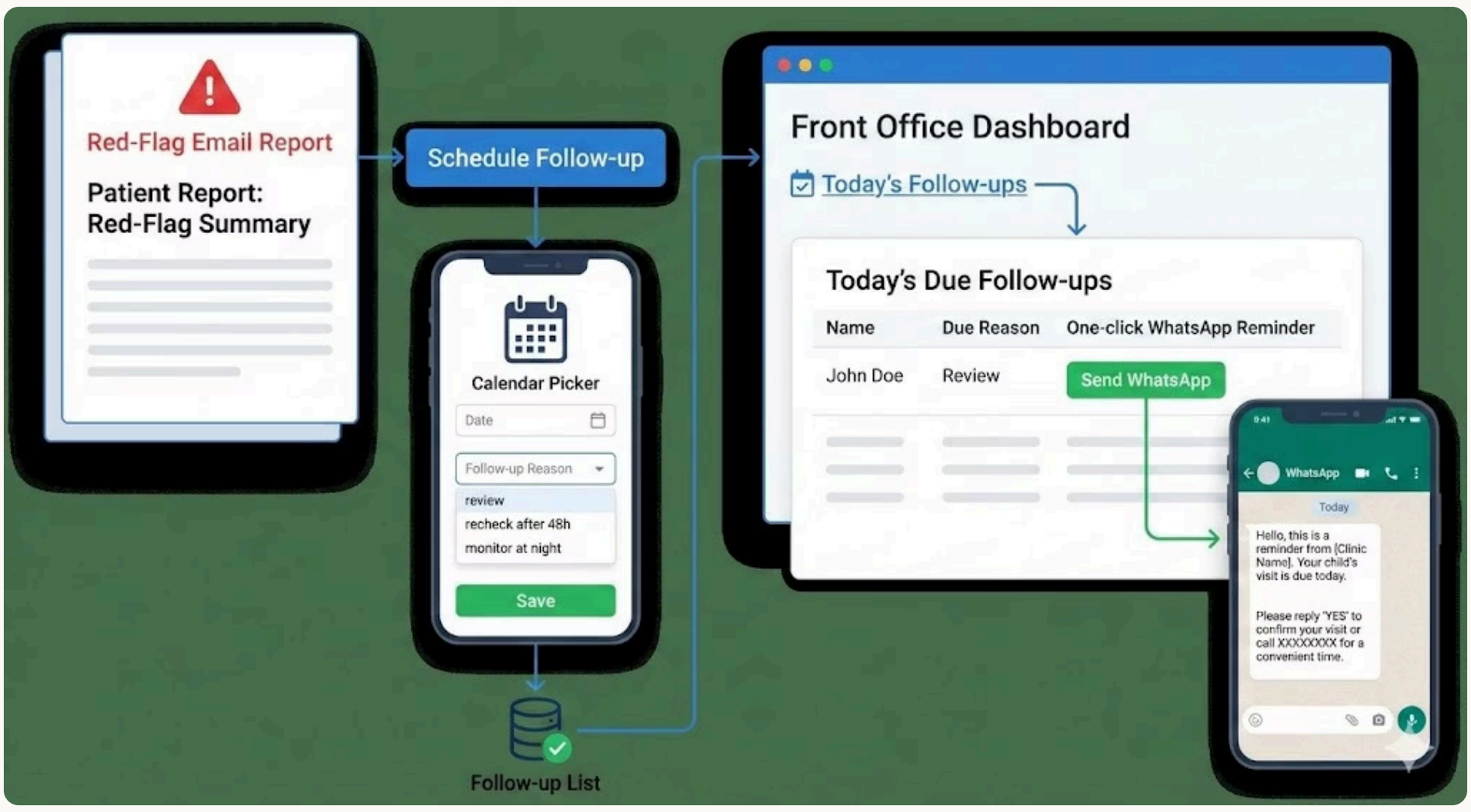
Brand principle:

- **No brand on patient screens**
- Brand appears only on doctor report/doctor education sheet, with a "supported by…" line.



D. Follow-up + reminder system

Added as part of the doctor email report and clinic staff workflow.



E. Reporting & measurement

Weekly/periodic reporting to brand team can include:

- field rep shares
- doctor opens
- completion reads (up to last page)

- patient video views
- certification completion

- follow-ups scheduled
- reminders sent

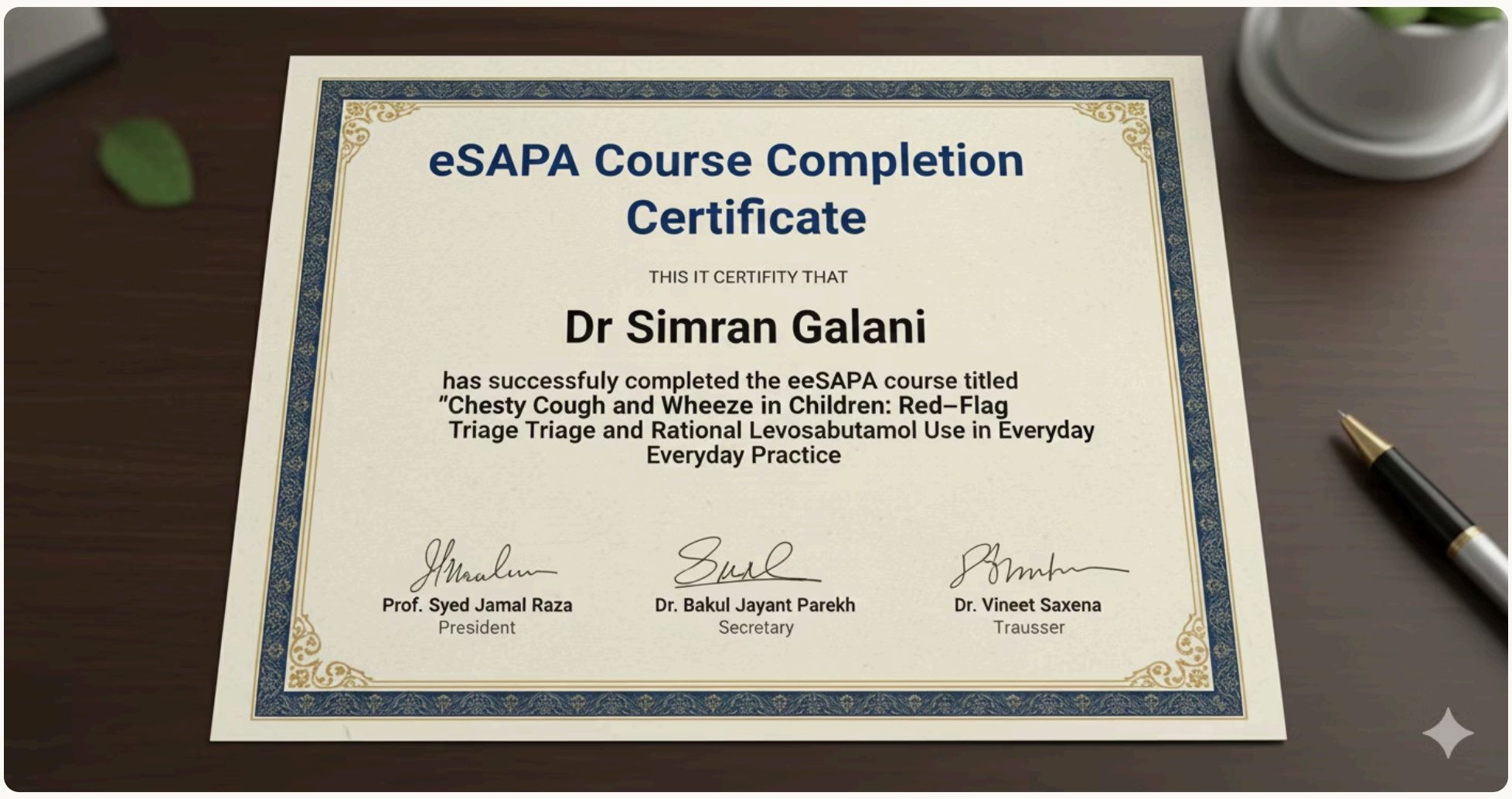
Doctor Certification Course

Course Title - “Chesty Cough and Wheeze in Children: Red-Flag Triage and Rational Levosalbutamol Use in Everyday Practice”

Module set (10 topics aligned to the publication calendar)

1. **When Levosalbutamol is the wrong answer**
   “The Nebuliser That Didn’t Work: When Levosalbutamol Is the Wrong Answer” — triage first; don’t assume all noisy breathing is wheeze/asthma.
2. **Viral wheeze vs emerging asthma: getting the first 3 episodes right**
   Early phenotype thinking, early pathway correctness, and avoiding mislabeling.
3. **Chesty cough in OPD: matching therapy to cough phenotype**
   “Chesty Cough in OPD: Matching Bronchodilator + Mucolytic/Expectorant to the Phenotype” — avoid “one syrup for every cough” habits.
4. **Route-of-delivery decisions in real clinics**
   “Syrup, Neb or Inhaler? Route-of-Delivery Decisions in Real Clinics” — choosing the right route and improving family acceptance.
5. **Breaking the short-acting reliever overuse cycle**
   “Three Visits in One Month: Breaking the Short-Acting β-Agonist Cycle” — frequent reliever use as a warning sign, not reassurance.
6. **Red-flag wheeze: when ‘asthma’ isn’t asthma**
   “When to Suspect TB, Bronchiectasis or Immunodeficiency” — stop escalation, widen diagnosis, and define referral thresholds.
7. **Bronchodilator safety in high-risk children**
   “Heart Disease, Arrhythmia and Hyperthyroidism” — safety, dose limits, and monitoring red flags.
8. **Not every tight chest is asthma**
   “Breathlessness, Anxiety and Dysfunctional Breathing” — avoid unnecessary bronchodilator escalation and recognize non-respiratory drivers.
9. **Wheeze in malnutrition: respiratory + nutrition red flags**
   Linking repeated wheeze/“bronchitis” patterns to nutrition/growth problems.
10. **Night/early-morning symptoms: control vs rescue thinking**
    “Night-Time and Early-Morning Symptoms: Are We Chasing Bronchospasm Instead of Fixing Control?” — structured follow-up, controller mindset, and preventing repeat exacerbation loops.

Course Certificate Printed and Distributed by Brand Field Reps In-Clinic



3) The 10-Publication Program Plan (April–December 2026) + mapping

How we will fit 10 publications into April–December

Option recommended (simple for field execution):

April 2026 = Kickoff month with 2 publications (Pub 1 + Pub 2)

May–Dec = one publication per month (8 more)

Total = **10 publications** across the April–Dec field window.

Publication calendar (10 case-study publications)

April 2026 (Kickoff – Publication 1)

Publication 1: *"The Nebuliser That Didn't Work: When Levosalbutamol Is the Wrong Answer"*

Parent video paired: *"Noisy Breathing in Children: When a Nebuliser Is NOT the Answer"*

Core behavior shift: "Not all noisy breathing = wheeze/asthma → triage red flags first."

April 2026 (Kickoff – Publication 2)

Publication 2: *"Under-5 Viral Wheeze vs Emerging Asthma: Getting the First Three Episodes Right"*

Parent video paired: *"My Child Wheezes With Every Cold: What This Really Means"*

Core behavior shift: phenotype thinking + early correct pathway.

May 2026

Publication 3: *"Chesty Cough in OPD: Matching Bronchodilator + Mucolytic/Expectorant to the Phenotype"*

Parent video paired: *"Chesty Cough: When Does Your Child Really Need a 'Cough + Breathing' Syrup?"*

Core behavior shift: avoid "one syrup for every cough" habit.

June 2026

Publication 4: *"Syrup, Neb or Inhaler? Route-of-Delivery Decisions in Real Clinics"*

Parent video paired: *"Syrup, Nebuliser or Inhaler: Which Breathing Medicine Is Right for My Child?"*

Core behavior shift: route clarity + better family acceptance.

July 2026

Publication 5: *"Three Visits in One Month: Breaking the Short-Acting β_2-Agonist Cycle"*

Parent video paired: *"Too Much 'Rescue' Medicine: What Frequent Nebs and Puffs Are Telling Us"*

Core behavior shift: "frequent reliever use = warning sign, not reassurance."

Publication calendar (continued)

August 2026

Publication 6: *"Red-Flag Wheeze: When to Suspect TB, Bronchiectasis or Immunodeficiency"*

Parent video paired: *"When 'Asthma' Isn't Just Asthma: Warning Signs in a Coughing, Wheezing Child"*

Core behavior shift: stop escalating bronchodilator; widen diagnosis.

September 2026

Publication 7: *"Bronchodilator Use in the High-Risk Child: Heart Disease, Arrhythmia and Hyperthyroidism"*

Parent video paired: *"Breathing Medicines in Children With Heart or Thyroid Problems: Why Doctors Are Extra Careful"*

Core behavior shift: safety, dose limits, monitoring.

October 2026

Publication 8: *"Not Every Tight Chest is Asthma: Breathlessness, Anxiety and Dysfunctional Breathing"*

Parent video paired: *"Tight Chest, Fast Breathing and Panic: When It's Not Just the Lungs"*

Core behavior shift: avoid unnecessary bronchodilator escalation.

November 2026

Publication 9: *"Wheeze in the Malnourished Child: Respiratory and Nutrition Red Flags"*

Parent video paired: *"Weak and Breathless: Why Nutrition Matters in Children With Cough and Wheeze"*

Core behavior shift: link repeated wheeze/bronchitis to growth/nutrition.

December 2026

Publication 10: *"Night-Time and Early-Morning Symptoms: Are We Chasing Bronchospasm Instead of Fixing Control?"*

Parent video paired: *"Night Cough, Early-Morning Wheeze: Why Control Matters More Than Just 'Rescue'"*

Core behavior shift: structured follow-up + controller thinking.

Additional Resources

4) Full Text sample and PDF sample for Publication 1

Complete case study content and educational materials - provided with this proposal as a separate document

Implementation & Execution

What the ASM does every month (execution playbook)

Each month per doctor:

01

Share / hand over Publication PDF

(printed or WhatsApp)

02

Point out the parent video QR/link

(so doctor asks staff to share)

03

Check adoption quickly (30 seconds)

- "Are parents scanning the poster?"
- "Are you receiving reports on email?"

04

Nudge one behavior

"Please ask reception to share the link to every parent."

9) Deliverables Summary (what we will produce/build)

Content deliverables

- **10 case study publications (PDF, print-friendly)** aligned to program themes
- **Parent education video library mapping** (10 used in Apr–Dec)
- **Doctor certificate course**

Tech deliverables

- Clinic-branded Cough Assistant links + multilingual screening forms
- Doctor report emails
- **Follow-up scheduler + staff reminder console**

Design deliverables

- Waiting-room QR poster
- External capability display board

Reporting deliverables

Weekly dashboards including:

- field shares, doctor opens, completion reads, patient video views
- screening forms filled, red flags incidence
- follow-ups scheduled, reminders sent

Commercials

- Cost: **One thousand five hundred rupees per clinic per year**.
- **Two thousand clinics** in the first year.
- This includes:
  - All content development with academy.
  - Build and hosting of the Cough Assistant microsite and tools.
  - Clinic white labelling and poster design.
  - Doctor reports and algorithms.
  - Basic technical support and reporting.
  - Online Certificate course for pediatricians